

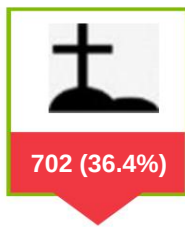
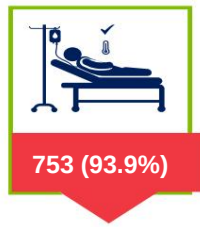
Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

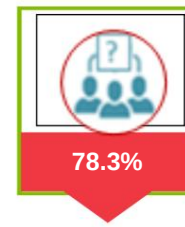
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°058/MVB_11/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (42)	• Haut-Uélé (1/13): Wamba • Ituri (26/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga and Ariwara. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
Reporting date	July 11, 2026
Publication date	July 12, 2026


 Total confirmed cases
for the 5 provinces

 Cumulative deaths
among confirmed cases
and case fatality rate

 Patients in
isolation -
hospitalization

 Cumulative number
of recoveries

 Suspected cases of
the day

 Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.
further investigation with a view to possible reclassification (Non-cases, probable cases,

1. HIGHLIGHTS

- In the past 24 hours, **53 new confirmed cases and 30 deaths (66.6% community transmission) have been recorded** in the provinces of Ituri (45 cases and 27 deaths) and North Kivu (8 cases and 2 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at **1,926** confirmed cases and **702** deaths, representing an overall case fatality rate of **36.4%**.
- **A new health zone affected in the province of Ituri, namely: Ariwara health zone.**
- **Ten (10) deaths** were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (4 at CTE Mongbwalu, 2 at CTE Elihya, 1 at CTE Rwampara, 1 at CT Sota and 1 at CT HGR Nia-Nia) and in North Kivu (1 at CTE Butembo).
- **Twelve (12)** patients with Ebola virus disease in Ituri province (3 at CTE CME, 2 at CTE Mongbwalu, 3 at CTE ISTM and 4 at CT Sota) have been declared cured after obtaining negative control tests, including one patient who was under clinical trial at CTE CME.
- Receipt of 18 motorcycles, donated by UNICEF, to strengthen the mobility of community actors in high-risk health areas
- Investigations are continuing in the provinces of Tshopo and Haut-Uele. It should be noted that all the cases in Tshopo (n=4) are imported cases from Nia-Nia. However, they have been assigned to the health zones of Kisangani where they were detected.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on **May 15, 2026**, the Ebola virus disease epidemic caused by **Bundibugyo ebolavirus** continues to evolve within a complex humanitarian and security context. Initially detected in the **Mongbwalu Health Zone (Ituri)**, it has spread to **three provinces (Ituri, North Kivu, and South Kivu)**, affecting **38**

Health Zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan are facilitating transmission and complicating response operations. Multisectoral coordination continues to limit the spread of the epidemic and strengthen public health interventions.

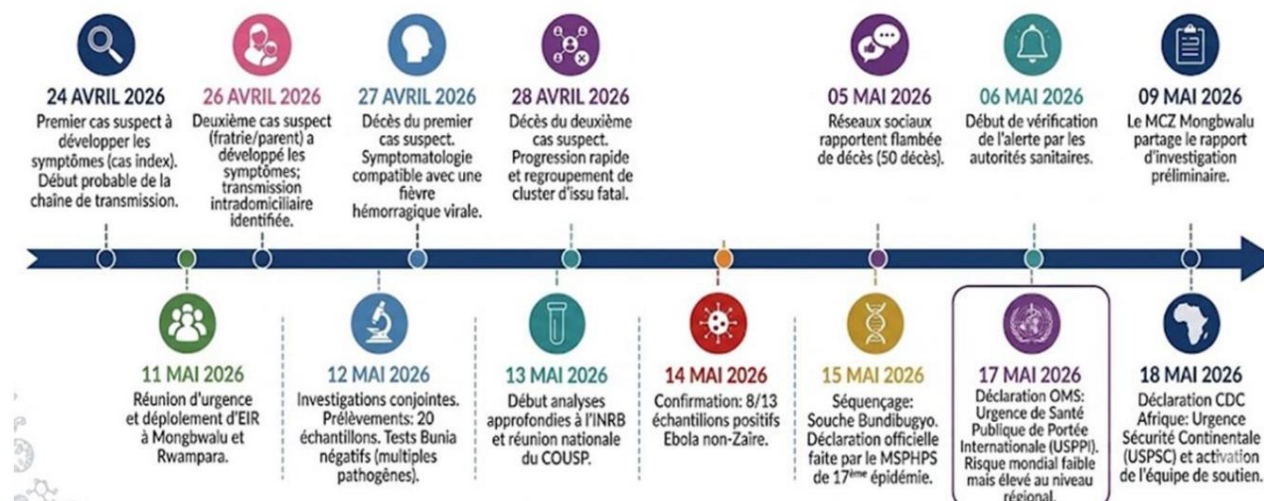


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 11, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of July 11, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri*	1745	601*	34.4%	26 out of 36 (72.2%)	45
North Kivu	173	97	56.1%	11 out of 34 (32.4%)	8
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Haut-Uélé	1	1	100.0%	1 out of 13 (7.7%)	0
Tshopo	4	2	50.0%	3 out of 23 (13.0%)	0
Total	1926	702	36.4%	42 out of 117	53

*Nine deaths in Ituri were recorded among patients with Ebola virus disease (previously confirmed cases) in Ebola treatment centers/CTs.

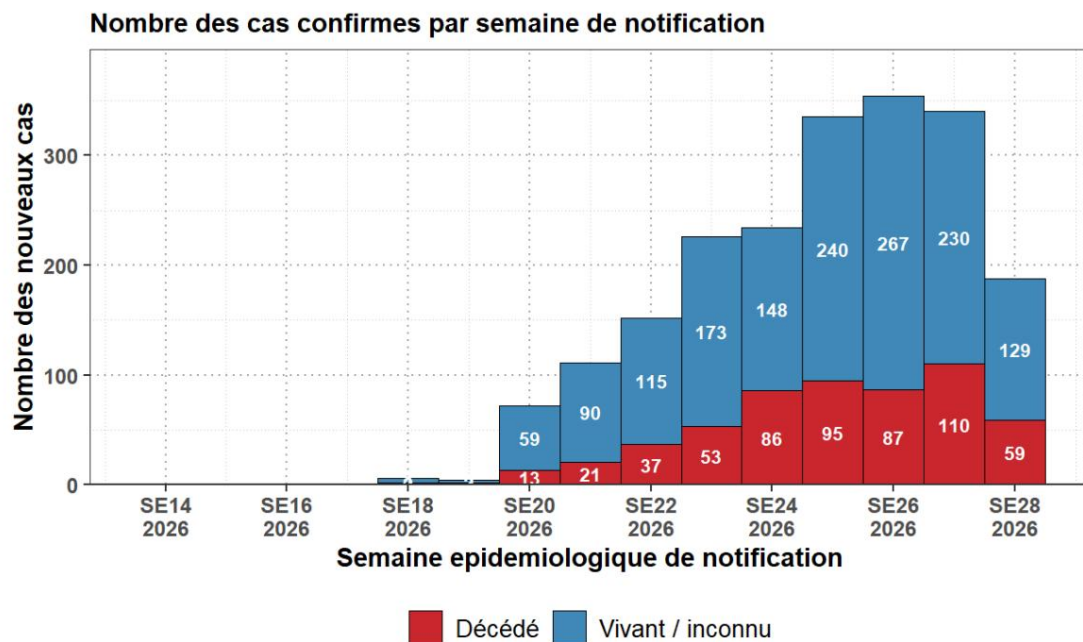
At the end of the day on July 11, 2026, a new health zone, Ariwara, in Ituri province, was affected within the last 24 hours. The cumulative number of affected health zones now stands at 42.

out of 117 (section 3.1). Since May 26, 2026, the province of South Kivu has not recorded any confirmed cases, suggesting an absence of recent transmission. As of July 7, 2026, the province of South Kivu officially passed the 42-day mark without notification of a new confirmed case, which corresponds to twice the maximum incubation period of the virus. This crucial step marks the end of the

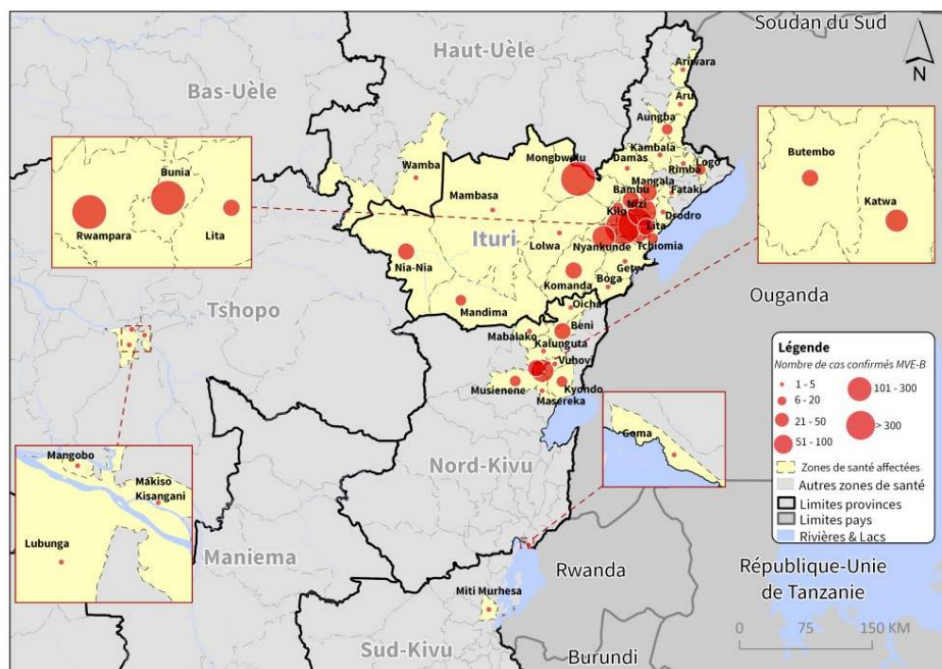
a period of enhanced surveillance and paves the way for the official declaration of the end of the epidemic in this province, subject to the maintenance of residual epidemiological vigilance.

3.2 Time-Based Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) confirmed continuous and increased community transmission in week 25. Compared to all weeks in which cases have been recorded since the beginning of the epidemic, weeks 25, 26, and 27...

continue to see growth in the number of reported cases, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 11, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 11, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1745	601	34.5%
Bunia	507	160	31.6%
Rwampara	393	90	22.9%
Mongbwalu	336	177	52.7%
Nyankunde	99	25	25.3%
Nizi	121	47	38.8%
Lita	48	19	39.6%
Komanda	27	8	29.6%
Bamboo	31	10	32.3%
Kilo			11.1%
Mangala	9	1	58.0%
Tchomia	50 19		21.1%
Damascus	5		0.0%
Aungba	6		33.3%
Rimba	4		0.0%
Aru	3		33.3%
Mambasa	5		80.0%
Logo	7		0.0%
Drodro	4		75.0%
Gety	1		0.0%
Kambala	2		100.0%
Nia-Nia	36	29	33.3%
Fataki	4		25.0%
Mandima	8	4	37.5%
Lolwa			100.0%
Boga			100.0%
Ariwara	11 1	0 2 0 1 4 0 3 0 2 12 1 3 1	100.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	173	97	56.1%
Butembo	44	21	47.7%
Katwa	71	47	66.2%
Blessed	30	20	66.7%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	7	1	42.9%
Goma		3	0.0%
Masereka	1	0	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene	3 1	1	20.0%
SOUTH KIVU	1 10 3	0 2 1	33.3%
Miti-Murhesa	3	1	33.3%

HAUT-UÉLÉ	1	1	100.0%
Wamba	1	1	100.0%
TSHOPO	4	2	50.0%
Makiso-Kisangani	2	2	100.0%
Mangobo	1	0	0.0%
Lubunga	1	0	0.0%
TOTAL	1926	702	36.4%

Over the past 24 hours, **53 new confirmed cases and 29 deaths (68.9% community transmission) have been recorded** in the provinces of Ituri (45 cases and 27 deaths) and North Kivu (8 cases and 2 deaths), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at 1,926 confirmed cases and 701 deaths, representing an overall case fatality rate of **36.4%**.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.9%** of cases (n=1,750) and **86.2%** of deaths (n=604), representing a crude case fatality rate of **34.5%**, making it the epicenter of the epidemic. The health zones of Bunia (507 cases), Rwampara (393 cases), Mongbwalu (336 cases), Nizi (121), and Nyankunde (99 cases) remain the most affected, accounting for approximately 83.2% of reported cases at the provincial level and 75.6% at the national level. In Nizi, the epidemic is currently progressing more rapidly than in the other zones.

Severity of the epidemic in North Kivu

A total of **173 confirmed cases and 97 deaths** have been reported in North Kivu province since the start of the outbreak, representing a **crude case fatality rate of 56.1%**. This high case fatality rate is concentrated in the health zones of **Katwa** (66.2%), **Beni** (66.7%), and **Butembo** (47.7%), while the other health zones account for only 32.1% of the case fatality rate. The three health zones with the highest number of reported cases are Katwa (n=71), followed by Butembo (n=44) and Beni (n=30).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding coordination meetings with technical and financial partners.
- Continuation of on-the-ground supervision activities of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 11, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)				202
New Alerts			0	1170
Total alerts for the day			12	1345
Alerts investigated			12	1179
Investigation rate			12 100.0%	87.6%
Alerts confirmed	Alive			158
	Deceased	175,462,637,471 73.24%	296,189,469 100.0%	70
Total suspected cases for today	166	56	6	228

During the day of July 11, 2026, a total of 1345 alerts were recorded in the provinces of Ituri, North Kivu and South Kivu, including 1170 new alerts and 202 alerts reported from the previous day, demonstrating the maintenance of active epidemiological surveillance in the affected provinces.

The majority of alerts were notified in North Kivu, with 696 alerts, or 100%, compared to 471 alerts, or 73.94%, in Ituri and 100% in South Kivu.

Of the alerts recorded, 1179 were investigated, representing an overall investigation rate of 87.6%. The investigation rate was 73.94% in Ituri and 100% in the other two provinces, highlighting the need to strengthen investigative capacities in Ituri province.

The investigations led to the validation of 228 suspected cases, including 158 living individuals (69.2%) and 70 deaths (30.7%). Ituri recorded the highest number of validated suspected cases, with 166 (35.2%), compared to 56 (8.0%) in North Kivu and 50% in South Kivu.

Table 4. Contact tracing of confirmed cases (1) as of July 11, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	10,450	7,886	75.5%
North Kivu	2,285	2084	91.2%
South Kivu*	0	0	N / A
Total	12,735	9,970	78.3%

*

End of follow-up for all contacts listed in the province of South Kivu.

As of July 11, **12,735** contacts were under follow-up in the provinces of Ituri and North Kivu. Of these, **9,970** had been seen, representing an overall follow-up rate of **78.3%**. Ituri had the largest number of contacts under follow-up, with 10,450 (75.5% of the total), and 7,886 contacts seen, also representing a follow-up rate of 75.5%. In North Kivu, 2,285 contacts were under follow-up, of which 2,084 (91.2%) had been visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 11, 2026

Province	Number of new contacts		Number of contacts had become suspects		Number of contacts made in the last 21 days	
	n		n		n	%
Ituri	364	75.7	13	68.4 %	294	80.3%
North Kivu		%		31.6%		19.7%
Total	117,481	24.3% 100.0%	6 19	100.0%	72,366	100.0%

The day ended on July 11 with **481 new contacts** listed, the majority in Ituri (364, 75.7%), while North Kivu listed 117 (24.3%). Among the contacts monitored, 19 developed symptoms consistent with the definition of a suspected case, including 13 in Ituri (68.4%) and 6 in North Kivu (31.6%). These contacts require immediate investigation and management in accordance with current procedures.

In addition, 366 contacts completed their 21-day follow-up period without developing symptoms consistent with the disease, including 294 in Ituri, or 80.3%, and 72 in North Kivu, or 19.7%.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure the early detection of secondary cases and the rapid interruption of any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

Ituri: Five (5) alerts were notified, including three (3) live alerts and two (2) deceased individuals. Among the live alerts, two (2) were validated, isolated, and referred to the CT/CTE, while one (1) was invalidated after reassessment; At the Nyamusasi PoC, a 26-year-old man, traveling from Tchomia to Kasenyi, presenting with fever, was validated, isolated, and transferred to the CT.

North Kivu: Two (2) alerts were notified. At the Kasitu PoC, a 16-year-old boy, of Congolese nationality, coming from Mongamba in Ituri to Butembo, presenting with a fever of 38.5 °C and headaches, was notified, investigated and referred to isolation at the Bundji CS.

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of 11 July 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (57/57)	100.0% (14/14)	100.0% (71/71)
Number of people who went through PoE/PoC testing,	90,389	53,705	144,094
% of travelers screened, % of	93.9%	98.0%	95.4%
travelers who washed their hands, % of	93.0%	98.0%	94.9%
travelers made aware of the issue	98.2%	98.0%	98.1%
Number of alerts notified	5	2	7
Number of validated live alerts	2		3
Number of lifeless bodies intercepted today	2	11	3
Number of problematic contacts intercepted today; %			0
of validated live alerts referred to CT/CTE; % of	0 100.0%	0 100.0%	100.0%
deceased bodies swabbed	50.0%	0.0%	33.3%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 227 samples collected and analyzed (**positivity 19.9%; n=45**).
- **North Kivu** : 123 samples collected and analyzed (**positivity 6.5%; n=8**).
- **South Kivu** : 6 samples collected and currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: Sixty-seven (67) households corresponding to the transmission of cases were targeted, and 42 were decontaminated within 48 hours, representing 62.7%. The twelve (12) healthcare facilities that housed a confirmed case were decontaminated within 48 hours, representing 100%. Furthermore, 175 out of 1,170 healthcare facilities had triage or pre-triage with a one-way flow system, representing 15.0%; 45 out of 678 healthcare facilities in high-risk health areas had received a complete IPC kit, representing 6.6%; 924 out of 1,640 healthcare providers had been trained in IPC/WASH Ebola, representing 56.3%; and 208 out of 1,170 healthcare facilities had water storage capacity for 48 hours, representing 17.8%.

4.5 EDS:

Ituri: Dignified and Safe Burials: Sixty-eight (68) deaths required attention, including 50 alerts for the day and 18 postponements. Thirty-six (36) EDS (Emergency Diagnosis and Discharge) procedures were carried out, six (6) deaths were buried without proper security by the families, and 26 were postponed, representing a 53% performance rate. Twenty-four (24) swabs (swabbing) were performed. A shortage of supplies for the EDS teams was reported in the Nizi Health Zone.

North Kivu: Seven (7) bodies were awaiting collection from the previous day, and 24 death alerts were received. In total, 24 bodies were recovered and secured, 26 dignified and secure burials were conducted, and five (5) bodies were postponed. The training of the EDS team in the Kalunguta Health Zone was completed.

4.6 Holistic care

Ituri:

- Nine (9) deaths were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (4 at CTE Mongbwalu, 2 at CTE Elikya, 1 at CTE Rwampara, 1 at CT Sota and 1 at CT HGR Nia-Nia).
- Twelve (12) patients with Ebola virus disease in Ituri province (3 at CTE CME, 2 at CTE Mongbwalu, 3 at CTE ISTM and 4 at CT Sota) have been declared cured after obtaining negative control tests, including one patient who was under clinical trial at CTE CME.
- Three ambulance runs were successfully completed, two from Lopa to the Nyankunde ISTM CTE and one from Sangaki, AS Mandro, to the Bunia General Hospital. Three ambulances were operational and one was inoperable due to a battery problem.
- By the end of the day on July 11, 936 hot meals had been served: 95 to suspected cases, 149 to Confirmed cases: 664 in PPLs and 28 in accompanying persons.

Patient movement:

- 94 new admissions were recorded in the CTE/CT/CI facilities of the three provinces, including 67 in Ituri, 21 in North Kivu and 6 in South Kivu, according to the latest available data. Discharges totaled 110 patients.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 11, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	544	201	24	769
Total admissions (24h)	67	21	6	94
Exits (24h)				
Deceased (Suspects/Confessions)	14	1	0	15
No case	42	33	0	75
Healed	12	0	0	12
Escapees (Suspects/Confessions)	8	0	0	8
Transferred to the HGR	0	0	0	0
Total outflows	76	34	0	110
Patients in isolation (End of Day),	535	188	30	753
including confirmed cases	227	21	0	248
(NC+AC) and suspected cases	308	201	30	539
Overall occupancy rate	87%	129.6%	66.6%	93.9%

In total, **753** patients were in isolation, including 248 confirmed cases and 539 suspected cases, for an overall bed occupancy rate of **93.9%**.

Table 8. Key indicators of mental health and psychosocial support activities as of July 11, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (18.2%)	0.0%	0
Confirmed cases currently being monitored that have received interventions SMSPS	98 (61.6%)	0.0%	0
New suspected cases that benefited from SMSPS interventions	29 (80.6%)	0.0%	2
Suspected cases under follow-up that have received interventions SMSPS	62 (37.3%)	0.0%	1 (100.0%)
Laboratory results have been announced.	12		1
Including positive ones	2	0	0
Number of children separated in daycare who benefited from SMSPS interventions	6	0	0
Number of children separated in the community who have benefited from interventions SMSPS	0	0	7
Number of accompanying persons at the CTE who benefited from SMSPS interventions	67	0	0
Number of PPLs who benefited from SMSPS interventions	26	1	0
Number of household and community members who benefited from SMSPS interventions	32	0	0
Number of EDS supported	2	0	0

4.7 Continuity of care

- 14/36 health zones shared the mapping of partners offering free healthcare in Ituri. • Implementation of effective free healthcare.
- Provision of essential medicines for the PMA and PCA in other health zones
- Support for non-cases (children, mothers and other vulnerable individuals) in continuity actions of services.
- Psychosocial support for affected individuals and their dependents

4.8 Risk communication and community engagement

Ituri: Two (2) advocacy meetings resulted in the engagement of 58 people (38 men and 20 women); 3,340 people were reached during home visits (1,479 men and 1,861 women); 2,649 people were reached during educational talks and focus groups (1,184 men and 1,465 women); 28 people were engaged during a community dialogue (23 men and 5 women); and 5,484 people were reached during 77 mass awareness campaigns (2,221 men and 3,263 women).

Community feedback: Twelve (12) pieces of feedback and informational messages were reported, of which ten (10) were recorded and two (2) clarified, representing 20% of the recorded feedback. The main concerns involved delays in the EDS teams, the management of community deaths, the discharge of a patient from the Rwampara Ebola Treatment Center, payment to service providers, and requests for funds for household decontamination.

Support for the pillars of the response

Surveillance: 97 community alerts were reported, including 70 active cases and 27 deaths, in the health zones of Bunia, Nyankunde, and Rwampara. Support was also provided for listing 11 contacts at the Gloria Clinic and for pre-listing around a contact case.

CREC: Twenty-four (24) community health workers in the Nizi health zone and 17 Red Cross volunteers in the Bunia health zone were briefed on Ebola Virus Disease (EVD); two radio programs and six public service announcements were produced and broadcast, 36 community radio stations were engaged, and 15 posters and IEC materials were distributed. Support was provided for the decontamination of three households in the Rwampara health zone, Shari health district.

North Kivu: 405 community actors reported their activities; 7,370 households were visited; 36,819 people were reached by prevention messages through door-to-door visits; 745 people were reached during dialogues and educational talks; 2,691 people were sensitized in public places; and 27 radio stations broadcast at least one communication format.

Community surveillance and feedback management: 212 alerts of suspected cases were reported by community actors; three (3) out of five feedback or infodemics were captured and resolved within 48 hours; 1,262 people were engaged in community actions; and one community incident was resolved within 48 hours. In the Katwa Health Zone, the CREC supported contact tracing, the transfer of a suspected case to the Ebola Treatment Center (ETC), and household preparation for decontamination. A mass communication campaign reached 63 students in Katwa, and 35 Community Action Committee (CAC) members were briefed in Mabalako.

Specific feedback: In the Kyondo health area, some residents are hesitant to send sick people to the Ebola Treatment Center (ETC), believing that the staff's lack of experience would inevitably lead to patient deaths. The chosen course of action is to strengthen community communication, particularly by sharing testimonials from people who have recovered.

4.9. Logistics

Ituri

- Receipt of 18 motorcycles, donated by UNICEF, to enhance the mobility of community actors in high-risk health zones; delivery of IPC, Surveillance, and PECH supplies to the Mangala and Lita health zones, as well as to the Sota Ebola Treatment Center; receipt of 10 tons of IPC and PECH supplies with support from WHO. In total, 25 vehicles and five (5) ambulances were made available to the various stakeholders. A total of 200 liters of fuel was distributed to the ambulances and operational vehicles.

North Kivu

Deployment of teams from the different pillars in the health zones; holding of the section meeting; packing of IPC kits and medicines for the management of cases at Mambangu Hospital; continuation of the unpacking of medicines received from UNICEF; and monitoring of the repair of mobile vehicle 103 at the Mutsora garage.

4.10. Security

- No new major security incidents were reported in the consolidated presentation of July 11, 2026. Vigilance remains heightened around PoE/PoC and support structures.
- Continued coordination with political, administrative and security authorities to ensure access for response teams to at-risk areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Continued community awareness-raising on the prevention of sexual exploitation and abuse.

Sixty-two (62) participants, including 45 men and 17 women, were briefed, bringing the total to 2,250 people briefed and who signed commitment forms, including 1,286 men and 964 women. A total of 107 people, including 54 women/girls and 53 men/boys, were sensitized, bringing the total to 12,872 people sensitized, including 6,944 men and 5,928 women. The PSEA activities were integrated into the Lita Health Zone, bringing the number of Health Zones covered to five: Bunia, Rwampara, Mongbwalu, Aru, and Lita.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2 Insufficient capacity of CTE/CT facilities and saturation in North Kivu (129.6%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3 Contact tracking performance not optimal (78.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (112 cases) HCW (including 35 deaths, case fatality rate 31.2%)	Urgent supply, strengthening of IPC training
8. Sample stock not yet analyzed (backlog) in the laboratory at North Kivu and late diagnosis	Delayed confirmation, high lethality (56.1%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10 Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (non-Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Reception of 18 motorcycles donated by UNICEF

**POUR TOUTE INFORMATION
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**Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare**